

Ochsner Health

Business Development Dossier

Industry	Healthcare System
Revenue	\$7.7B
Employees	40,000
Ideal Customer Profile Score	92

ICP SCORING MODEL



ICP = Signals + Revenue + Employees + Fit (max 100)

Signals (0-35)	Top 4 signals scored; each = base(60%) + recency bonus <30d: 40% <90d: 30% <180d: 20% older: 10%
Revenue (0-25)	\$500M-\$10B sweet spot -> 70-100% of 25 pts Below \$500M -> pro-rata x 0.5 Above \$10B -> 60%
Employees (0-20)	2,000+ preferred (70-100%) 500-2K partial (50-70%) Below 500 -> pro-rata x 0.3
Fit (0-20)	High-fit types: reorg, transformation, M&A, hiring, funding, partnership -> 5 pts ea (max 12) + diversity 2/type (max 8)

Prepared March 26, 2026

McChrystal Group

Leadership Advisory & Organizational Effectiveness

SECTION 1

Organization Snapshot

Legal Name	Ochsner Clinic Foundation (d/b/a Ochsner Health)
Headquarters	New Orleans, Louisiana
Founded	1942
Ownership	Nonprofit 501(c)(3) organization
Footprint	Primary operations across Louisiana, Mississippi, Alabama, and the Gulf South; 47 hospitals, 370+ health and urgent care centers

SECTION 1b

Company Overview

Ochsner Health is the largest nonprofit healthcare provider in the Gulf South, operating 47 hospitals and 370+ health and urgent care centers with \$7.7 billion in operating revenue and 40,000+ employees. Under new CEO Pete November (since November 2025), Ochsner is integrating Rush Health Systems (7 hospitals), affiliating with Allegiance Health Management (12+ rural hospitals), deploying system-wide EPIC, and pivoting to value-based care. The system recently reshuffled its executive team with a new COO (Dr. Timothy Riddell, January 2026) and promoted Chief People & Culture Officer (Tracey Schiro).

SECTION 2

Financial Health & Growth Stage

Understanding Ochsner Health's financial position and trajectory is critical to framing any engagement conversation.

- Revenue: FY 2024 operating revenue \$7.7B (up 5.7% from 2023)
- Operating income: \$157.8M (2.0% margin) in 2024, up from \$104M (1.4%) in 2023 - significant improvement but thin margins
- Credit: Moody's A3, stable outlook
- Pressures: Rising labor costs, Medicaid reimbursement volatility, hurricane/disaster risk, integration complexity, rural hospital economics
- Tailwinds: Dominant Gulf South market position, digital health leadership, growing managed care component, population growth

SECTION 3

Leadership Team Profiles

The following contacts are recommended as primary outreach targets - champion-level leaders who feel the organizational pain daily and can sponsor McChrystal internally.

Timothy Riddell, MD - EVP & Chief Operating Officer [PRIORITY TARGET]

Family medicine physician; 28 years within Ochsner (since residency 1997). Previously Regional CEO for Northshore/Mississippi Gulf Coast; CMO for multiple regions

Outreach rationale: Brand new in COO role, inheriting 47 hospitals and 370+ clinics during CEO transition and massive integration challenge. Needs operating model support now

Tracey Schiro, MBA - EVP, Chief People & Culture Officer [PRIORITY TARGET]

MBA University of New Orleans. Joined as AVP Compensation/Benefits; progressive career through VP HR Ops, SVP & CHRO

Outreach rationale: Leads people and culture across rapidly expanding system integrating multiple acquired organizations with different cultures

Additional leadership team members relevant to the engagement:

Pete November - President & CEO

Attorney (equity partner at Alston & Bird 14 years); then SVP General Counsel, EVP Digital Health, EVP & CFO at Ochsner. Named AHA Board of Trustees (Nov 2025)

Scott Posecai, CPA - EVP, CFO & Treasurer

CPA. Previously oil and gas finance (accounting, treasury, international tax)

Denise Basow, MD - EVP & Chief Digital Officer

Leads innovationOchsner, virtual care, remote patient monitoring, chronic disease management

SECTION 4

Organizational Culture & Structure

Culture and structure are often where McChrystal Group finds the deepest leverage. The following signals indicate how Ochsner Health operates today and where friction exists.

Organizational Structure

Large multi-regional system with regional CEOs and CMOs. Recently reorganized with new COO (Riddell) and executive team under new CEO (November). Mix of fully owned, merged (Rush Health), and affiliated (Allegiance) entities creating a federated structure

Culture Signals

Glassdoor: 3.7/5.0 (2,198 reviews); 74% recommend; Culture 3.8/5.0

Great Place to Work: 82% say it's a great place (May 2025)

Positives: Good benefits, team environment, leadership engagement, employee programs

Concerns: Short staffing, starting pay rates, last-minute schedule changes (common healthcare complaints)

M7 Platform: Deployed across all 47 hospitals' nursing units by end 2025, filling 2,000+ shifts/week automatically

Integration challenge: Multiple merged/affiliated entities (Lafayette General, Rush Health, Allegiance) each with legacy cultures needing unification under one Ochsner identity

SECTION 5

Recent News & Trigger Events

The following developments from the past 18 months create urgency and provide natural conversation entry points for engaging Ochsner Health.

- 1. 2025-11-01 - Pete November becomes CEO; executive team reshuffled
CEO transition - highest-impact organizational moment. New CEO establishing vision and team dynamics for 40,000-person system
- 2. 2025-12-18 - Allegiance Health affiliation LOI signed - 12+ rural hospitals in LA/MS
Federated operating challenge - how to coordinate entities you don't own. Classic Team of Teams
- 3. 2026-01-01 - Timothy Riddell named EVP & COO, replacing 13-year COO Mike Hulefeld
New COO inheriting massive scope during peak transformation. Needs operating model clarity immediately
- 4. 2026-02-01 - M7 workforce platform fully deployed across all 47 hospitals
Technology deployment at scale creates change management and cross-unit coordination demands
- 5. 2025-05-06 - Vitality partnership launches Medicare Advantage engagement solution
Value-based care innovation requiring cross-functional coordination

SECTION 6

McChrystal Fit Assessment

CEO transition (Nov 2025), new COO (Jan 2026), active affiliation (Allegiance), clear buyers identified (Riddell, Schiro), revenue >\$7B, organizational urgency (integration + transformation + leadership transition simultaneously)

Primary Problem

New CEO leading 40,000-person system through simultaneous challenges: integrating multiple acquired/affiliated entities, transitioning to value-based care, deploying system-wide technology, and establishing new leadership team - all while maintaining clinical quality across 47 hospitals

Best McChrystal Capability Fit

Team of Teams for federated health system; leadership alignment for newly appointed executive team; organizational design for multi-entity coordination; cross-functional alignment for value-based care transformation

Likely Objections to Engagement

We're a nonprofit with 2% margins - can we afford boutique advisory?
We have an internal Leadership Institute
Healthcare is different from military

Competitive Advisory Landscape

McKinsey/Deloitte likely on strategy and IT transformation
Vizient or Chartis Group for clinical operations
Huron Consulting for financial/operational performance
No competitor for 'operating model for federated, multi-entity health system with new leadership'

SECTION 7

Conversation Entry Points

These questions are designed for a Senior Partner's first conversation. Each opens a thread that naturally leads to McChrystal Group's capabilities without a direct pitch.

1. You're building a federated healthcare system - entities you own, entities you've merged, entities you're affiliating with - all needing to coordinate without single command-and-control. How are you thinking about the operating model that holds that together?

Team of Teams challenge in healthcare terms

2. Pete's first year as CEO with a new COO, new executive team structure, and Allegiance coming online - what rhythm are you establishing for how this leadership team operates?

Operating rhythm and leadership alignment

3. You've deployed M7 across 47 hospitals and you're scaling digital medicine - but the technology only works if the organizational model supports it. How is clinical leadership adapting to a data-driven, value-based operating model?

Technology-to-organization bridge

Recommended Meeting Framing

'We work with large, complex organizations navigating multiple integrations, leadership transition, and operating model transformation simultaneously.'

SECTION 8

Brand Insights & Market Positioning

Brand strategy reveals organizational priorities and coordination challenges. The following analysis connects Ochsner Health's market positioning to potential McChrystal engagement opportunities.

Brand: Dominant Gulf South healthcare brand; #1 hospital in Louisiana by U.S. News for 14 consecutive years. Limited direct competition.

Evolution: From 'Louisiana's best hospital system' to 'Gulf South's leading integrated health system' - now encompasses hospitals, clinics, health plans, digital health, and research.

Threats: Rapid expansion risks diluting quality promise; rural affiliations may create 'two-tier' perception; HCA/Tenet competing in adjacent markets.

Investments: \$132M Neuroscience Institute (opening late 2026); digital health platform driving national recognition.

McChrystal connection: Delivering consistent 'Ochsner experience' across 47 hospitals, 370 clinics, and 12+ affiliated rural hospitals with different ownership/culture requires exactly the cross-organizational coordination McChrystal builds.

SECTION 9

Deep McChrystal Group Fit Analysis

This is the strategic case for pursuing Ochsner Health. It synthesizes findings from all previous sections into a comprehensive engagement thesis with specific fit dimensions, expected outcomes, stakeholder mapping, and a phased pursuit strategy.

9a. Fit Dimensions

Dim 1: New CEO Leadership Team Alignment - Pete November's team has been together <5 months. Quality of their operating rhythm will determine transformation success. McChrystal builds team operating models; Korn Ferry coaches individuals.

Dim 2: Federated Health System Operating Model - Mix of owned, merged, and affiliated entities. Can't command-and-control an entity you don't own - need shared consciousness and trust-based cooperation. McChrystal's core capability. Chartis designs governance; McChrystal makes it work in practice.

Dim 3: Value-Based Care Transformation - Fee-for-service to outcomes-driven changes how every function operates. Cross-functional alignment at scale. McKinsey designs VBC strategy; McChrystal solves the organizational design problem.

Dim 4: Multi-Entity Cultural Integration - Lafayette General, Rush Health, now Allegiance - each with distinct cultures. McChrystal builds common language, operating rhythms, and trust networks across boundaries.

9b. Cumulative Case

CEO transition (Nov 2025) -> exec reshuffle -> COO change (Jan 2026) -> Allegiance LOI (Dec 2025) -> EPIC deployment -> VBC pivot -> M7 across 47 hospitals. All converging in Q1 2026 during new CEO's first 6 months = maximum receptivity window.

Revenue: Phase 1: \$500K-\$800K (exec alignment) | Phase 2: \$1.5M-\$2.5M (federated system operating model) | Phase 3: \$400K-\$750K/year | Total: \$2.4M-\$4.05M

9c. Enterprise Issues

1. New executive team operating rhythm not established (<5 months together)
2. Federated system coordination challenge (owned + merged + affiliated)
3. COO transition during maximum operational complexity
4. Value-based care operating model immaturity
5. Rural health extension without operational control (Allegiance)
6. Digital transformation organizational strain (EPIC + M7 + AI simultaneously)
7. Post-acquisition cultural integration debt (Lafayette General, Rush Health)

9d. Expected Outcomes

1. Executive team operating model - decision cycle time reduced 30-40%
2. Federated coordination framework - Allegiance integration milestones on schedule
3. Regional leadership alignment - clinical variation reduced 20-30%
4. VBC cross-functional coordination - care gap closure improved 15-20%
5. Technology change management - adoption rates >85% within 6 months of deployment
6. Cultural integration measurement - merged facility engagement trending to system averages

9e. Key Stakeholders

CEO Office

Leader: Pete November

Title: President & CEO

Relevance: Executive sponsor

Operations

Leader: Timothy Riddell, MD

Title: EVP & COO

Relevance: PRIMARY TARGET - new in role, maximum complexity

People & Culture

Leader: Tracey Schiro, MBA

Title: EVP CPO

Relevance: PRIORITY TARGET - culture integration mandate

Finance

Leader: Scott Posecai, CPA

Title: EVP CFO

Relevance: Budget holder, 35 years at Ochsner

Digital Health

Leader: Denise Basow, MD

Title: EVP CDO

Relevance: Technology transformation stakeholder

9f. Opportunity Thesis

Why Now: New CEO's first 6 months = maximum receptivity. New COO <3 months in. Federated model (Allegiance) creating unprecedented coordination challenge. Leaders are most open to advisory input before default patterns establish.

Why McChrystal: Traditional consulting assumes hierarchical authority. McChrystal builds coordination without authority - shared consciousness across organizational boundaries. No other firm specializes in this for healthcare.

Phased Engagement: Phase 1 (3-4 months, \$500K-\$800K): Exec team alignment | Phase 2 (8-12 months, \$1.5M-\$2.5M): Federated system + VBC operating model | Phase 3 (\$400K-\$750K/year): Ongoing advisory + Leadership Institute integration

Pursuit Map: Thread 1: COO Riddell via operational complexity. Thread 2: CHRO Schiro via culture integration. Thread 3: CEO November via AHA Board/healthcare CEO networks.